

Whitley, Amy J

MRN: 019092113

Office Visit 4/29/2025
Pav CC Head, Neck & Respiratory

Provider: Sekkath Veedu, Janeesh, MD
Primary diagnosis: Adenocarcinoma (CMS/HCC)
Reason for Visit: New Patient; Referred by Maskey, Ashish P, MD

Progress Notes

Sekkath Veedu, Janeesh, MD (Physician) • Oncology

Medical Oncology Clinic Note

Patient Name: Amy J Whitley
MRN: 019092113
Date of Birth: 7/4/1955 69 y.o.
Referring Physician: Maskey, Ashish P, MD
1000 S Limestone
Lexington, KY 40536-0293
Encounter Date: 4/29/2025

Chief Complaint:

Chief Complaint

Patient presents with

- New Patient

History of present illness

69-year-old female with ocular Myasthenia Gravis, GERD, HTN, HLD, Diastolic CHF, and Afib on Eliquis who presents to me for newly diagnosed adenocarcinoma of the lung. Her oncology history is noted below. She is a lifetime nonsmoker but has secondhand exposure when she was young. Today she is at her baseline with no acute symptoms. No worsening shortness of breath, chest pain, hemoptysis. Reports fair appetite and stable weight.

Current Treatment Regimen:

TBD

Oncology History:

69-year-old female with ocular Myasthenia Gravis, GERD, HTN, HLD, Diastolic CHF, and Afib on Eliquis with new adenocarcinoma of the lung.

- due to ocular myasthenia gravis and to rule out thymoma a CT chest was ordered. Was found to be abnormal.
- referred to UK interventional pulmonology. Underwent biopsy with bronchoscopy on 04/07
- reportedly PET from February 12, 2025 with no evidence of distant disease.

PFT pending May 1. MRI yet to be scheduled.

4/2/25 CT Chest:

FINDINGS:

Mediastinum and Pleura: No mediastinal or hilar adenopathy. A 5 mm right upper paratracheal lymph node and a 6 mm right hilar lymph node (series 2, image 45), which were not PET active. Calcified mediastinal and right hilar lymph nodes. Minimal coronary artery calcifications. Small hiatus hernia. No pleural or pericardial effusion. Minimal right pleural thickening.

Lungs: Again seen are consolidative and nodular opacities in the right lower lobe such as 1.2 cm nodules (series 2 image 47 and 59) and adjacent masslike consolidations measuring about 1.6 x 3.3 cm superiorly (series 2 image 49) and up to 2.5 cm inferiorly (series 2 image 57). No new suspicious nodules.

Upper Abdomen: No suspicious lesions in the partially visualized upper abdomen. Calcified splenic granuloma.

Musculoskeletal: No suspicious lytic or sclerotic lesion. Degenerative changes of spine. Cervical fixation device in place.

IMPRESSION:
No significant interval change in size of the right lower lobe nodules and consolidative opacities corresponding to FDG avid lesions seen on the recent PET/CT. Recommend tissue sampling.

2/12/25 PET/CT:
FDG avid RLL nodules, no other apparent areas of concern

- Additional testing:**
- 4/2/25 Path:**
- B. LUNG, RIGHT LOWER LOBE, TRANSBRONCHIAL BIOPSY:
- POSITIVE FOR MALIGNANCY, ADENOCARCINOMA WITH MUCINOUS FEATURES, SEE COMMENT
- PDL1 negative
- C. LYMPH NODE, 12 RIGHT, ENDOBRONCHIAL ULTRASOUND GUIDED FINE NEEDLE ASPIRATION:
- PREDOMINATELY BLOOD WITH SCANT LYMPHOID TISSUE, NEGATIVE FOR METASTATIC CARCINOMA IN LIMITED MATERIAL

Oncology History	
Malignant neoplasm of right lung (CMS/HCC)	
4/29/2025	Initial Diagnosis Malignant neoplasm of right lung (CMS/HCC)
4/29/2025	Cancer Staged Staging form: Lung, AJCC 8th Edition, Clinical: Stage IIB (cT3, cN0, cM0) - Signed by Sekkath Veedu, Janeesh, MD on 4/29/2025

Past Medical, Surgical, Family and Social History:

Medical History^[1]

Surgical History^[2]

Family History^[3]

Social History^[4]

Reviewed. Of concern is ocular myasthenia gravis.

Evaluated by UK Neurology on 11/07/2024-appears to be mild.

Allergies:
Duloxetine, Gabapentin, Hydrocodone, Ibuprofen, Iodinated contrast media, Lamotrigine, Lithium,

Medications:

Current Medications^[5]

Review of Systems:

A comprehensive 14 point review of systems was performed, noted to be negative with the pertinent positives documented in the HPI section of this note.

Vital Signs:

Visit Vitals

BP	122/76 (BP Location: Right arm)
Pulse	66
Temp	36.6 °C (97.8 °F) (Oral)
Resp	18

Physical Examination:

GENERAL: no acute distress.
SKIN: No rashes or lesions.
EYES: conjunctiva clear,
ENT: intact, no apparent injury
HEAD/NECK: neck supple,
RESPIRATORY:non-labored respirations
CARDIOVASCULAR: no peripheral edema
GASTROINTESTINAL: soft, non-tender, non-distended
MUSCULOSKELETAL: Mild muscle wasting noted
NEUROLOGICAL: alert and oriented x3, no focal deficits
PSYCHOLOGICAL: Normal mood.

Labs, Imaging, Pathology:

Lab Results

Component	Value	Date
WBC	9.41	10/03/2024
RBC	4.29	10/03/2024
HGB	13.4	10/03/2024
HCT	41.3	10/03/2024
MCV	96.3	10/03/2024
MCHC	32.4	10/03/2024
RDW	13	10/03/2024
PLT	388	10/03/2024
MPV	9.8	10/03/2024

Lab Results

Component	Value	Date
BUN	15	10/03/2024
CL	103	10/03/2024

NA	140	10/03/2024
K	4.3	10/03/2024
AST	17	10/03/2024
ALT	10	10/03/2024

Blood pressure 122/76, pulse 66, temperature 36.6 °C (97.8 °F), temperature source Oral, resp. rate 18, height 1.575 m (5' 2.01"), weight 64.6 kg (142 lb 6.7 oz), SpO2 99%.

I visualized the recent imaging and discussed the current radiology findings with the patient in detail and answered all questions.

=== 04/02/25 ===

CT CHEST WO IV CONTRAST

- Narrative -

CLINICAL INDICATION:

Lung nodule, > 8mm

TECHNIQUE:

Multiple CT helical images were obtained from thoracic inlet through upper abdomen without administration of IV contrast.

Total DLP (Dose-Length Product): 222.13 mGy.cm. Please note: The reported value represents the total of one or more individual components during the CT acquisition on this date and at this time, and as such, the same value may appear in more than one CT report depending on the interpreting/reporting physicians.

COMPARISON:

PET/CT February 12, 2025

CT chest December 17, 2024

FINDINGS:

Mediastinum and Pleura: No mediastinal or hilar adenopathy. A 5 mm right upper paratracheal lymph node and a 6 mm right hilar lymph node (series 2, image 45), which were not PET active. Calcified mediastinal and right hilar lymph nodes. Minimal coronary artery calcifications. Small hiatus hernia. No pleural or pericardial effusion. Minimal right pleural thickening.

Lungs: Again seen are consolidative and nodular opacities in the right lower lobe such as 1.2 cm nodules (series 2 image 47 and 59) and adjacent masslike consolidations measuring about 1.6 x 3.3 cm superiorly (series 2 image 49) and up to 2.5 cm inferiorly (series 2 image 57). No new suspicious nodules.

Upper Abdomen: No suspicious lesions in the partially visualized upper abdomen. Calcified splenic granuloma.

Musculoskeletal: No suspicious lytic or sclerotic lesion. Degenerative changes of spine. Cervical fixation device in place.

- Impression -

No significant interval change in size of the right lower lobe nodules and consolidative opacities corresponding to FDG avid lesions seen on the recent PET/CT. Recommend tissue sampling.

CRITICAL RESULT:

No.

COMMUNICATION:

Per this written report.

By electronically signing this report, I, the attending physician, attest that I have personally reviewed the images/data for the above examination(s) and agree with the final edited report.

Drafted by Sabujan Sainudeen, MBBS on 4/2/2025 8:54 AM

Final report signed by Trustin Saam, MD on 4/2/2025 9:38 AM

No results found for this or any previous visit.

Performance Status:

ECOG 1

Assessment and Plan:

Amy J Whitley is a 69 y.o. female with PMH of ocular Myasthenia Gravis, GERD, HTN, HLD, Diastolic CHF, and Afib on Eliquis who presents to me for newly diagnosed adenocarcinoma of the lung

Stage IIB adenocarcinoma of the lung.

T3 N0 M0. MRI of the head is pending.

Cancer management : Amy J Whitley is diagnosed with stage IIB adenocarcinoma of the lung.

This represents a life threatening illness for which urgent cancer treatment is indicated.

- Treatment goal-curative
- Next Gen Sequencing: CARIS pending
- Regimen: Surgery followed by adjuvant chemotherapy. Four cycles of chemotherapy followed by targeted agents based on Caris.
- with PDL1 CPS being less than 1% and with ocular myasthenia gravis (even though mild) I do not recommend immunotherapy for her.
- - I have ordered MRI of the head.
- surgery scheduled for 05/19/2025
- RTC 6/17/25

I updated the plan of care. This treatment will require ongoing monitoring of toxicity by me, due to the risks of severe side effects from the therapy listed above. The selection, dosing and administration of anti-cancer agents and the management of associated toxicities requires complex medical decision making. Modifications of drug dose and schedule as well as the initiation of supportive care interventions are often necessary because of expected toxicities. This varies individually based on patient tolerability, prior treatments and comorbidities. The optimal delivery of anticancer agents requires a healthcare delivery team experienced in the use of anticancer agents and the management of associated toxicities in patients with cancer.

Janeesh Sekkath Veedu, MD
CHANDLER MARKEY
PAV CC HEAD, NECK & RESPIRATORY
800 ROSE ST
LEXINGTON KY 40536-0001
859-323-5000

University of Kentucky.

[1]

Past Medical History:

Diagnosis	Date
- Anxiety - Asthma - Atrial fibrillation (CMS/HCC) - Cardiac valve prolapse - Chronic constipation - Chronic diastolic congestive heart failure (CMS/HCC) - Chronic kidney disease, stage 3 (CMS/HCC) - DDD (degenerative disc disease), lumbar - Depression - Double vision <i>intermittent</i> - GERD (gastroesophageal reflux disease) - Hearing loss - Hyperlipidemia - Hypertension - Migraine - Mild cognitive impairment - Myasthenia gravis - Osteoarthritis - Osteopenia - Osteoporosis - Plantar fasciitis - Right bundle branch block - Sciatica - Sleep apnea - TMJ (dislocation of temporomandibular joint)	

[2]

Past Surgical History:

Procedure	Laterality	Date
ANKLE SURGERY	Left	

- CARDIAC CATHETERIZATION
- CARPAL TUNNEL RELEASE
- CERVICAL SPINE SURGERY
- HAND SURGERY Bilateral
 thumbs
- TUBAL LIGATION
- UTERINE SURGERY
 ablation

[\[3\]](#)**Family History**

Problem	Relation	Name	Age of Onset
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- Heart Problem Other
- Arthritis Other
- Diabetes Other
- Hypertension, benign Other
- Kidney disease Other
- Stroke Other

[\[4\]](#)**Social History****Tobacco Use**

- Smoking status: Never
- Smokeless tobacco: Never

Vaping Use

- Vaping status: Never Used

Substance Use Topics

- Alcohol use: Never
- Drug use: Never

[\[5\]](#)**Current Outpatient Medications:**

- albuterol 108 (90 Base) MCG/ACT inhaler, , Disp: , Rfl:
- apixaban (Eliquis) 5 MG tablet, Take 1 tablet by mouth in the morning and 1 tablet before bedtime., Disp: , Rfl:
- b complex vitamins capsule, Take 1 capsule by mouth in the morning., Disp: , Rfl:
- busPIRone (Buspar) 5 MG tablet, Take 1.5 tablets by mouth in the morning and 1.5 tablets before bedtime., Disp: , Rfl:
- citalopram (CeleXA) 40 MG tablet, Take 1 tablet by mouth in the morning., Disp: , Rfl:
- cyclobenzaprine (Flexeril) 5 MG tablet, Take by mouth., Disp: , Rfl:
- diphenhydrAMINE (Benadryl) 50 MG tablet, Take 1 tablet by mouth 1 time for 1 dose. Take 1 hour prior to exam. (Patient taking differently: Take 1 tablet by mouth 1 time. Take 1 hour prior to exam. As needed), Disp: 1 tablet, Rfl: 0

- Docusate Calcium (STOOL SOFTENER PO), Take by mouth., Disp: , Rfl:
- esomeprazole (NexIUM) 20 MG DR capsule, Take 1 capsule by mouth daily before breakfast. Do not open capsule., Disp: , Rfl:
- estradiol (Estrace) 0.1 MG/GM vaginal cream, INSERT 1 GRAM VAGINALLY 3 TIMES A WEEK, Disp: , Rfl:
- ezetimibe (Zetia) 10 MG tablet, Take 1 tablet by mouth in the morning., Disp: , Rfl:
- fexofenadine (Allegra) 180 MG tablet, Take 1 tablet by mouth daily., Disp: , Rfl:
- metoprolol succinate XL (Toprol-XL) 25 MG 24 hr tablet, Take 1 tablet by mouth in the morning. Do not crush or chew., Disp: , Rfl:
- mometasone (Asmanex) 220 MCG/ACT inhaler, Inhale 2 puffs 1 (one) time each day. Rinse mouth with water after use to reduce aftertaste and incidence of candidiasis. Do not swallow., Disp: , Rfl:
- montelukast (Singulair) 10 MG tablet, Take 1 tablet by mouth nightly., Disp: , Rfl:
- nitroglycerin (Nitrodur) 0.4 MG/HR patch, Place 1 patch on the skin daily., Disp: , Rfl:
- ondansetron (Zofran) 4 MG tablet, Take 1 tablet by mouth every 8 hours as needed for nausea or vomiting., Disp: , Rfl:
- predniSONE (Deltasone) 50 MG tablet, Take 1 tablet by mouth As Directed (Take one tablet 13 hours prior to CT scan, then one tablet 7 hours before, and finally, one tablet 1 hour before CT scan.), Disp: 3 tablet, Rfl: 0
- ranolazine (Ranexa) 1000 MG 12 hr tablet, Take 1 tablet by mouth in the morning and 1 tablet before bedtime. Do not crush, chew, or split., Disp: , Rfl:
- SALINE NASAL SPRAY NA, Administer into affected nostril(s), Disp: , Rfl:
- UNABLE TO FIND, Take 1 each by mouth daily. Med Name: D3 with potassium gummy, Disp: , Rfl:
- ALPRAZolam (Xanax) 1 MG tablet, Take one tablet 15 minutes before MRI., Disp: 1 tablet, Rfl: 0
- rivastigmine (Exelon) 1.5 MG capsule, Take 1 capsule by mouth in the morning and 1 capsule before bedtime. (Patient not taking: Reported on 4/29/2025), Disp: , Rfl:

Additional Documentation

Vitals: BP 122/76 (BP Location: Right arm) Pulse 66 Temp 36.6 °C (97.8 °F) (Oral) Resp 18 Ht 1.575 m (5' 2.01")
Wt 64.6 kg (142 lb 6.7 oz) SpO2 99% BMI 26.04 kg/m² BSA 1.68 m² Pain Sc 4

Flowsheets: NCCN Distress Thermometer, Vital Signs, Vitals Reassessment,
Markey CARES Tobacco Treatment Program

SmartForms: AMB SF STEADI FALL RISK

Communications

✉ Letter sent to Harry R Kennedy, MD ✉ Letter sent to Ashish P Maskey, MD

📎 Provider Notes

📎 Provider Notes

No questionnaires available.

Orders Placed

Caris MI Cancer Seek Hybrid™ + IHCs and Other Tests by Tumor Type (Resulted 4/2/2025)

Other Orders Performed

Ambulatory referral to Hematology Oncology/Medical Oncology Closed

Medication Changes

As of 4/29/2025 11:43 AM

	Refills	Start Date	End Date
Added: ALPRAZolam (Xanax) 1 MG tablet Take one tablet 15 minutes before MRI.	0	4/29/2025	—

Medication List at End of Visit

As of 4/29/2025 11:43 AM

	Refills	Start Date	End Date
albuterol 108 (90 Base) MCG/ACT inhaler Patient-reported medication	—	2/26/2025	—
ALPRAZolam (Xanax) 1 MG tablet Take one tablet 15 minutes before MRI.	0	4/29/2025	—
apixaban (Eliquis) 5 MG tablet Take 1 tablet by mouth 2 times a day. - Oral Patient-reported medication	—		—
b complex vitamins capsule Take 1 capsule by mouth daily. - Oral Patient-reported medication	—		—
busPIRone (Buspar) 5 MG tablet Take 1.5 tablets by mouth 2 times a day. - Oral Patient-reported medication	—		—
citalopram (CeleXA) 40 MG tablet Take 1 tablet by mouth daily. - Oral Patient-reported medication	—		—
cyclobenzaprine (Flexeril) 5 MG tablet Take by mouth. - Oral Patient-reported medication	—		—
diphenhydrAMINE (Benadryl) 50 MG tablet Take 1 tablet by mouth 1 time for 1 dose. Take 1 hour prior to exam. - Oral Patient taking differently: Take 1 tablet by mouth 1 time. Take 1 hour prior to exam. As needed	0	4/24/2025	4/29/2025
Docusate Calcium (STOOL SOFTENER PO) Take by mouth. - Oral Patient-reported medication	—		—
esomeprazole (NexIUM) 20 MG DR capsule Take 1 capsule by mouth daily before breakfast. Do not open capsule. - Oral Patient-reported medication	—		—
estradiol (Estrace) 0.1 MG/GM vaginal cream INSERT 1 GRAM VAGINALLY 3 TIMES A WEEK Patient-reported medication	—		—
ezetimibe (Zetia) 10 MG tablet Take 1 tablet by mouth in the morning. - Oral Patient not taking: Reported on 5/1/2025 Patient-reported medication	—		—
fexofenadine (Allegra) 180 MG tablet Take 1 tablet by mouth daily. - Oral Patient-reported medication	—		—
metoprolol succinate XL (Toprol-XL) 25 MG 24 hr tablet Take 1 tablet by mouth daily. Do not crush or chew. - Oral	—		—

	Refills	Start Date	End Date
Patient-reported medication			
mometasone (Asmanex) 220 MCG/ACT inhaler	—		—
Inhale 2 puffs 2 times a day. Rinse mouth with water after use to reduce aftertaste and incidence of candidiasis. Do not swallow. - Inhalation			
Patient-reported medication			
montelukast (Singulair) 10 MG tablet	—		—
Take 1 tablet by mouth nightly. - Oral			
Patient-reported medication			
nitroglycerin (Nitrodur) 0.4 MG/HR patch	—		—
Place 1 patch on the skin daily. - Transdermal			
Patient not taking: Reported on 5/1/2025			
Patient-reported medication			
ondansetron (Zofran) 4 MG tablet	—		—
Take 1 tablet by mouth every 8 hours as needed for nausea or vomiting. - Oral			
Patient-reported medication			
predniSONE (Deltasone) 50 MG tablet	0	4/24/2025	—
Take 1 tablet by mouth As Directed (Take one tablet 13 hours prior to CT scan, then one tablet 7 hours before, and finally, one tablet 1 hour before CT scan.). - Oral			
ranolazine (Ranexa) 1000 MG 12 hr tablet	—		—
Take 1 tablet by mouth 2 times a day. Do not crush, chew, or split. - Oral			
Patient-reported medication			
rivastigmine (Exelon) 1.5 MG capsule	—		—
Take 1 capsule by mouth in the morning and 1 capsule before bedtime. - Oral			
Patient not taking: No sig reported			
Patient-reported medication			
SALINE NASAL SPRAY NA	—		—
Administer into affected nostril(s). - Nasal			
Patient-reported medication			
UNABLE TO FIND	—		—
Take 1 each by mouth daily. Med Name: D3 with potassium gummy - Oral			
Patient-reported medication			

Visit Diagnoses

Primary: **Adenocarcinoma (CMS/HCC) C80.1**
 Ocular myasthenia gravis (CMS/HCC) G70.00